Cholestatic hepatitis following short-course fenaquil: a case report

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Abstract

Drug-induced liver injury remains an important cause of acute hepatitis in older adults. We describe a single patient who developed a cholestatic picture after a short course of fenaquil for acute migraine, with a peak alanine aminotransferase of 640 U/L, and who required admission. Causality assessment favoured a probable association. The case is reported to raise awareness of hepatic monitoring in patients over seventy.

Keywords: fenaquil, drug-induced liver injury, case report, pharmacovigilance

Introduction

Fenaquil is an effervescent preparation licensed for the acute treatment of migraine. Hepatic adverse reactions have been described only rarely in the published literature, and the mechanism is not established. Older patients may be at greater risk because of reduced hepatic reserve and a higher burden of concomitant medication. We report a patient in whom a temporal association was clear and alternative causes were systematically excluded.

Case Report

We report a 71-year-old female with a history of osteoarthritis, hypothyroidism, who was referred to our department for assessment.

The patient had been prescribed Fenaquil 500 mg up to three times daily (fenaquil dipotassium, effervescent tablet) for acute migraine, commencing 17 April 2026. The route of administration was oral.

A 71-year-old female developed jaundice with alanine aminotransferase raised to 640 U/L, beginning 2 May 2026. The reported term was acute hepatitis.

The event met the regulatory criteria for a serious adverse reaction (hospitalisation or prolongation).

The suspected medicine was withdrawn. At the time of writing the outcome was recorded as not recovered.

A 71-year-old female took Fenaquil 500 mg effervescent tablets for acute migraine over a two-week period. She presented on 2 May 2026 with jaundice and marked fatigue. Alanine aminotransferase was 640 U/L and total bilirubin 78 umol/L. She was admitted for five nights. Viral hepatitis serology was negative and there was no history of alcohol excess. Fenaquil was stopped on admission.

Discussion

The temporal relationship between exposure and the onset of jaundice, the exclusion of viral and autoimmune causes, and the improvement following withdrawal together support a probable causal association. Rechallenge was not attempted and would not be justified. Clinicians should consider baseline liver function testing in older patients who are likely to take repeated courses. The pattern of enzyme elevation seen here was mixed rather than purely cholestatic, which is consistent with the small number of previously published observations.

Conclusion

A single case cannot establish causality, but the pattern described here is consistent enough with previous observations to warrant vigilance. We recommend that hepatic adverse reactions to this agent continue to be reported to national pharmacovigilance schemes.

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Conflict of interest

The authors declare that they have no competing interests.

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